



Quality Assurance and Oversight

Resident Care Manual

8.1 Internal Audit Process.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

The Merrill Gardens internal audit process consists of an evaluation of operational processes to help make sure the services provided in each community meet the standards set by residents, regulatory agencies and Merrill Gardens. The process is designed to assist communities with identifying their areas of compliance and needed improvement to help facilitate achievement of desired Operational, Sales and Marketing and Health Services goals. The goal is to be proactive and establish practices that meet standards in addition to correcting practices that fail to meet standards before problems occur.

Definitions

Sample

Representative part from a larger whole or group of items when presented for inspection shown as evidence of compliance.

Review

An evaluation of an operation's compliance to preset policies, procedures and governing regulations.

Category

A grouping or listing of related standards.

Standard

Component of or actual policy procedure and governing regulations.

Regulatory Agency

A local, State or Federal oversight agency that sets and supervises the operation, execution, and administration of their standards, policies and regulations.

Action Plan

Detailed plan for resolving noncompliant issues that includes:

- Action to be taken.
- Person(s) responsible to take that action.
- Timeline for completion.
- Verification that an action has been completed successfully.

Guidance

Resources

RESOURCES AND INFORMATION ABOUT STANDARDS

There are several resources available to each community and they should be used to answer any questions or address any concerns before an audit takes place. The following is an overview of resources:

- **Support Staff** will assist the community when requested to do so. Individual department heads should be encouraged to first discuss their concerns with their General Manager.
- **Department Written Materials.** Wherever possible, each of the standards listed on the audit report sheet will reference a policy. This will make it very easy to go straight to the needed information.
- **State, Local and Federal Regulations.** A copy of your State, Local or Federal regulations can be obtained from the particular agency in question. Health Services staff can assist you in obtaining this material; or you can visit the agency's web site.

The audit tool is used to document areas of focus related to systems implementation and on-going compliance, opportunities identified and the suggested next steps necessary for compliance. A copy of the audit tool will be sent to each community so there will be no confusion on what standards each auditor will be reviewing or sampling. When an auditor arrives to conduct an audit, the audit will be their focus. If there are any questions that need to be answered or any clarification needed on any of the listed standards, this must be brought to the attention of the appropriate support staff. **It is very important to seek clarification on an audit standard before the day of the actual audit.**

REGULATIONS VS. COMPANY POLICY:

Every effort has been made to create company policies that are in line with State, Local or Federal regulations. There are times when a company policy meets and / or goes beyond these regulations. At those times the company policy takes precedence. In the event you come across a company policy that doesn't meet a particular regulatory standard, you must meet the regulatory standard and bring this to the attention of your VPO as soon as possible. This happens most often when an agency amends a particular regulation. **Each community must remember that when a company fails to meet their own policies this is cause for a citation from regulatory agencies.**

Audit Frequency

Communities	Frequency	Auditor
Stabilized Communities	Annually	Director of Health Services, Health Services Manager
New Communities	Three Months after receiving initial license	Director of Health Services, Health Services Manager, VP of Resident Care
Focus Communities- Increase in turnover in key department positions, or poor regulatory history.	Twice per year	Director of Health Services, Health Services Manager, VP of Resident Care

Additional audits may be completed at any time to determine the compliance status of a community.

How Acceptable Compliance is Determined

The audit worksheet is divided into A, B, and C categories. Each category contains a list of Standards that are reviewed or sampled for compliance. The overall percentage of compliance will be tabulated based on the number of Standards in each category and the number of noncompliant findings. For example, if there are 20 Category B Standards, three noncompliant findings would result in 85% compliance.

DEFINITIONS FOR EACH CATEGORY:

- **Category A - High priority** - issues that would likely result in a negative life/safety, hazardous, or significant financial impact if not corrected.
- **Category B - Medium priority** - issues that would likely have a negative, or critical impact on the quality of life, expose the community to regulatory or legal action, or some financial impact if not corrected.
- **Category C - Low priority** - issues with paperwork not in compliance, with regulations or policies which would not likely have a negative outcome on health / safety of residents / staff, or financial impact if not corrected.

ACCEPTABLE COMPLIANCE:

- 100% of Category A Standards reviewed or sampled must be in compliance.
- 85% of Category B Standards reviewed or sampled must be in compliance.
- 75% of Category C Standards reviewed or sampled must be in compliance.

NONCOMPLIANT FINDINGS

All noncompliant findings, in all categories, will result in the creation of an action plan and timeline for correction (see Action Plan and Timeline below).

If a misfiled document which causes a noncompliant Standard can be produced by the exit meeting, the noncompliant Standard will be changed to acceptable compliance.

FAILED AUDIT

A Community will fail an internal audit if they do not meet acceptable compliance in any of the three categories. Given that 100% compliance is required in Category A, any noncompliant finding of Standards in Category A will result in a failed audit. Findings in Categories B and C that fall below the acceptable compliance percentage as stated above will result in a failed audit. When the overall results of an audit produce a failed audit, the auditor will work closely with the community to help ensure and confirm correction.

The Standards as outlined by the audit process are standards that are applicable to all team members, and are not just for the department heads in each community. Although a department head may be missing, the General Manager (GM) must seek to ensure that the department team members continue with the established action plans and corrective actions. In the absence of the General Manager and the department head, the responsibility will then fall to the Regional VP of Operations (VPO), with assistance from Seattle support staff to help ensure compliance.

FIRST FINDING

When an auditor cites a community for a noncompliant finding, this will result in an action plan and timeline for correction.

REPEAT FINDINGS (Second, Third or Fourth)

When an auditor cites a community for the same noncompliant finding that has previously occurred within the previous two years, this is considered a repeat finding. Site visit reports and regulatory inspections are reviewed for the purpose of determining repeat issues, with particular emphasis on Category A Standards. Once again, this may not result in an overall audit failure but will result in an action plan and timeline for correction.

REPEAT AUDIT FAILURE

When a community fails the same departmental audit that was previously failed within the last audit cycle, this is considered a repeat failure. In addition to specific actions related to the noncompliant findings applied as noted above, the failed audit will be treated as a Category A High Priority finding.

POSSIBLE EXCEPTIONS TO REPEAT FINDINGS

When there is a new management team member (General Manager, Resident Care Director, Garden House Director, etc.) it is very important to review previous audits with them. The length of time between the new manager's employment and the next audit will determine if they will be accountable for a repeat finding if the first was cited prior to their employment. This time frame is usually within the first 90 days of employment.

ACCOUNTABLE EMPLOYEE

Generally, the department head will be the accountable employee for the audit findings in their department. In the case of repeat findings and repeat audit failures the General Manager will be an accountable employee in addition to the department head. As previously noted, the standards as outlined by the audit process are standards that are applicable to all team members and are not just for the department heads in each community. Although a department head may be missing, the General Manager must seek to ensure that the department team members continue with the established action plans and corrective actions. In the absence of the General Manager and the department head, the responsibility will then fall to the VPO, with assistance from the Seattle support team to help ensure compliance.

Maintaining Compliance

The expectation is that each community will maintain compliance throughout the year. Methods used to maintain compliance are self-audits and detection, and site visits.

SELF-AUDITS AND DETECTION

Communities are expected to use the audit tool to self-audit as a regular practice and not just for audit preparation. The communities should promptly contact the VPO when Category A concerns are self-identified. The Seattle support team will be available to assist you in implementing an acceptable action plan for resolving the noncompliant issues per the acceptable timeframe for correction. Self-reported deficiencies are not a free pass when audit time comes, but will be considered based on the audit findings. The auditor will exclude the specific incident identified through self-detection when determining compliance during the audit.

SITE VISITS

We may randomly audit for compliance of Category A standards while conducting site visits. Non-compliant Category A standards found during a site visit shall be treated as though they were part of a formal audit for purposes of determining repeat findings. The normal appeal process will apply.

Pre-Audit Tasks

The following are completed in advance of, and in preparation for, the audit:

1. An annual schedule of audits will be developed by the Seattle department head prior to the start of the audit season.
2. The current audit tool is provided to the community at least two weeks in advance of the audit.
3. The auditor contacts the GM to set a date for an audit visit for the first round of audits. Other departmental audits may be unannounced.
4. Previous audit and recent survey reports are reviewed by the auditor prior to the audit visit.

Tasks Performed During the Audit Visit

Following are the guidelines to be followed during the audit visit:

1. Entrance Conference

On the day of the audit, the auditor meets with the GM and appropriate department head for an entrance conference.

- Brief descriptions of the audit process, records needed and estimated exit are discussed.
- Recent inspection reports are reviewed if applicable.
- Plan is made for reaching the GM if any significant concerns are identified.
- GM is asked if there are any concerns that they want a focus on.

2. Resident Sample Selection

- The resident sample should include at least 10% of the assisted living residents, with a minimum sample size of 10.
- When a standard is found to be out of compliance, the sample size should be increased by an additional 10% (minimum of 10) for that standard to determine whether the size of the sample caused the standard to be out of compliance.
- If the standard remains out of compliance after the sample size is increased, the GM is to audit all residents for that standard and submit the written results as part of the plan of action.
- If a team member standard remains out of compliance after the sample size is increased, the BOD is to audit all care team member records for that standard and provide the written results to the GM to submit as part of the action plan.
- Sampled residents are to include all residents at a service level of 10 or higher and anyone receiving care from a private caregiver/outside agency. In addition, any Garden House resident with a service level of less than 5 should be included in the sample.

3. Team Member Sample Selection

- The team member sample should include at least 10% of the care team members, with a minimum sample size of five. In communities with a Garden House, an additional minimum sample of five should be included.

- When a standard is found to be out of compliance, the sample size should be increased by an additional 10% (minimum of five) for that standard to determine whether the size of the sample caused the standard to be out of compliance.
- If the standard remains out of compliance after the sample size is increased, the GM is to audit all care team files for that standard as part of the plan of action.
- Sampled team members are to include any new hires in the past three months and anyone who has given notice to terminate employment.

4. Audit

- The auditor completes the departmental audit tool.
- Any Category A findings will be reported to the GM as soon as practical.
- The GM will be updated on the status of the audit periodically.
- The auditor will confirm the exit appointment with the GM as soon as they have an idea of when they will complete the audit.

5. Written Audit Results

- At the completion of the audit, the auditor will provide the completed written audit to the GM. The auditor will review the audit findings with the GM and the associated department head at the completion of the audit. If the GM or department head are not available at the completion of the audit, the auditor will review the audit findings via teleconference.
- The GM will be informed of the appeal process for any disputed findings.
- If the audit is not finalized by the exit meeting, the completed portions will be discussed and an appointment will be set for a follow up call to review the finalized audit will be made. The audit will be finalized within three days.
- The completed audit report will be emailed to the GM for development of the Action Plan.
- Standards determined to be "met" require no action plan. The GM is to write an Action Plan for any items determined to be "not met" and submit it to the auditor within 14 days.
- The VPO will be notified within 24 hours of any failed audits and items uncorrected from previous audit (repeat items).

Action Plans and Timelines

All action plans and timelines for noncompliance are outlined on the Action Plan section of the audit report. The timelines are specified in the Audit Action Guidelines and begin upon receipt of the final written audit report. The timelines are as follows:

- Category A noncompliant standards to be corrected within 14 days.
- Category B noncompliant standards to be corrected within 30 days.
- Category C noncompliant standards to be corrected within 45 days.

The plan will include tasks needed to correct the deficiency, assigned responsibility, and a due date for correction. The GM and community department head sign the plan to indicate agreement with and a commitment to follow through with it.

Refer to the box below and the following descriptions for a summary of the action plan layout.

Merrill Gardens
Health Services Action Plan and Correction Summary

Community: [Specify on Resident Audit D1] Auditor: [Specify on Audit Summary] Date: [Chart Audit K1]

Observation		Action	By Whom	Due Date	Date Completed
Standard					
Policy					
Category/Code					Completion Confirmed By

Standard:	The reference number for each Standard.
Priority:	A, B or C will appear in the space below the standard number to indicate the category or priority of the finding
Observation:	The auditor, if need be, will elaborate on their findings.
Action:	The GM will outline what action is to be taken to correct the findings and prevent recurrence.
By Whom:	The GM will list the responsible team member(s) that must complete, or oversee the completion of the action item.
Due Date:	All due dates will be in accordance with the Audit Action Guidelines unless exception is granted by the VPO.
Completion:	When an item has been corrected, the responsible team member(s) will note the date completed and initial. The GM will notify the auditor of completion prior to/or on each due date. At the completion of all action items the GM will scan and email the completed audit to the auditor and VPO for their records.
Confirmed:	Depending on the scope and priority of the action that needs to be taken, an auditor may request another support person to confirm that all of the corrections have been made and consistently maintained.

HOW TO APPEAL NONCOMPLIANT FINDINGS

An audit is a snap shot of your operation at the time the auditor is conducting the audit. There are times when a community may disagree with audit findings. A progressive three step appeals process has been put in place to address these concerns. This process is as follows:

1. Discuss your concerns with the auditor during the exit meeting or when you receive your written audit report. Be prepared to present documented information to support the appeal. Indicate on the report any noncompliant standards you wish to appeal. The auditor will discuss the issue with the VP of Resident Care and reach a conclusion.
2. If no agreement can be reached in the first step within three days, the appeal must be presented to the VPO. Once again be prepared to present documented information to support the appeal. The VPO will bring this to attention of the VP of Resident Care for discussion.
3. If the issue is not resolved by using steps one or two within 10 days, the VPO and VP of Resident Care will compile their findings and recommendation(s) to be presented to the VP of Operations. The decision at this level will be final and binding for all parties concerned.

If the appeal is unsuccessful, a community will be held responsible for the original action plan and timeline. Additionally, the community remains responsible for all other corrective action plans.

Tasks Performed After the Audit Visit

1. The GM implements corrections based on the Action Plan.
2. The auditor emails the entire audit tool with action plan to the VPO, and VP of Resident Care.
 - The VP of Resident Care reviews the audit and forwards the audit to the VP of Operations, VP of Health Services and President.
3. The auditor will follow up with the GM to verify due dates for corrections are met.
 - The GM will contact the auditor weekly during the Plan of Action to monitor progress.
 - Deviations from the Plan of Action will be discussed and the auditor will notify the VPO.
4. When the GM has verified the completed corrections, evidence of the completed action plan is sent to the auditor, VPO and the VP of Resident Care.
 - Category A noncompliant standards require revisit by support staff within 60 days to verify correction.
 - Category B and C noncompliant standards will be verified by the GM and the verified Action Plan sent to the auditor.
 - The VP of Resident Care will monitor completion of audit corrections.

Merrill Gardens Health Services Audit Guidelines

Definitions:

- (A) High priority -usually an issue that would likely result in a negative life/safety, hazardous, or significant financial impact if not corrected.
- (B) Medium priority -usually an issue that would likely have a negative, or critical impact on the quality of life, expose the community to regulatory or legal action, or some financial impact if not corrected.
- (C) Low priority-- usually an issue with paperwork not in compliance with regulation or policy which would not likely have a negative outcome on health / safety of residents / staff or financial impact if not corrected.

Audit acceptability performance standards for 2017 are:

- All (A) items sampled must 100% to be acceptable.
- All (B) items sampled must be 85% or above to be acceptable.
- All (C) items sampled must be 75% or above to be acceptable.

RESIDENT: ARE AUDIT

[illegible]

x = Complete
o = Not found

l = Incomplete
na = Not applicable
* = Multiple items per standard

RESIDENT CARE AUDIT

Community:		Date:									
TREND		RESIDENT APARTMENT NUMBER									
ITEM	Standard	101	102	103	104	105					
HEALTH SERVICES											
MEDICATION/TREATMENT RECORDS											
MARs match current MD orders & meds (SAMPLE)	A5										
Omission of initials <3%	B27*										
Doses not given circled with reason on back	B27*										
PRNs have clear parameters for use	C45										
PRNs documented on back with results	B27*										
Meds are stored in original or Pharmacy packaging	B26										
Daily narc count is completed and accurate	B25										
CONSULTS/MISCELLANEOUS											
Plan & visit notes for resident arranged services	B31*										
Outside provider Info on Service Plan	B31*										
Registration and Information form (129) &	B31*										
Acknowledgement and Indemnification (132) &	B31*										
Waiver & Release & Insurance (133) OR	B31*										
Copy of License, Insurance & Contract for Agency	B31*										
ASSESSMENT/CARE PLANS											
Resident Evaluation & Service Plan at move in	A2*										
Required services provided (include NSA approaches)	A4										
Signed and dated by resident or legal representative	B12*										
Points are scored correctly	B13										
Reflects change in condition	B15*										
Reevaluated at least every 6 mo (annual for IND)	B15*										
Reevaluated upon return from HLOC	B16										
Indicated level matches billing	C38										
Monthly review done	C39										
Outside Services identified	B22										
Side rail/assistive device evaluation documented	B17										
Negotiated Service Agreement-if applicable	A1										
Resident meets MG & State guidelines for residency											
GARDEN HOUSE - ADDITIONAL ITEMS											
GH Notification of Risks and Benefits signed	A3										
Mini Mental Sataus Exam completed	C41										
Physical Self Maintenance Scale completed	C42										
ADDITIONAL ITEMS											
Oxygen is secured. Signs are posted.	B23										
Copy of current vaccination records for pets	C47										

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 o = Not found
 i = Incomplete
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[illegible]

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Merrill Gardens Health Services Audit Guidelines

Definitions:

- (A) High priority -usually an issue that would likely result in a **negative life/safety, hazardous, or significant financial impact** if not corrected.
- (B) Medium priority -usually an issue that would likely have a **negative, or critical Impact on the quality of life, expose the community to regulatory or legal action, or some financial Impact** if not corrected.
- (C) Low priority-- usually an issue with **paperwork** not in compliance with regulation or policy which would **not likely have a negative outcome** on health / safety of residents / staff or financial impact if not corrected.

Audit acceptability performance standards for 2017 are:

- All (A) items sampled must 100% to be acceptable.
- All (B) items sampled must be 85% or above to be acceptable.
- All (C) items sampled must be 75% or above to be acceptable.

[illegible]

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RESIDENT CARE AUDIT

Community:		Date:									
ITEM	TREND	101	102	103	104	105	RESIDENT APARTMENT NUMBER				
HEALTH SERVICES											
MEDICATION/TREATMENT RECORDS											
MARs match current MD orders & meds (SAMPLE)	A5										
Omission of initials <3%	B27*										
Doses not given circled with reason on back	B27*										
PRNs have clear parameters for use	C45										
PRNs documented on back with results	B27*										
Meds are stored in original or Pharmacy packaging	B26										
Daily narc count is completed and accurate	B25										
CONSULTS/MISCELLANEOUS											
Plan & visit notes for resident arranged services	B31*										
Outside provider info on Service Plan	B31*										
Registration and Information form (129) &	B31*										
Acknowledgement and Indemnification (132) &	B31*										
Waiver & Release & Insurance (133) OR	B31*										
Copy of License, Insurance & Contract for Agency	B31*										
ASSESSMENT/CARE PLANS											
Resident Evaluation & Service Plan at move in	A2*										
Required services provided (include NSA approaches)	A4										
Signed and dated by resident or legal representative	B12*										
Points are scored correctly	B13										
Reflects change in condition	B15*										
Reevaluated at least every 6 mo (annual for IND)	B15*										
Reevaluated upon return from HLOC	B15*										
Indicated level matches billing	B16										
Monthly review done	C38										
Outside Services identified	C39										
Side rail/assistive device evaluation documented	B22										
Negotiated Service Agreement-if applicable	B17										
Resident meets MG & State guidelines for residency	A1										
GARDEN HOUSE - ADDITIONAL ITEMS											
GH Notification of Risks and Benefits signed	A3										
Mini Mental Sataus Exam completed	C41										
Physical Self Maintenance Scale completed	C42										
ADDITIONAL ITEMS											
Oxygen is secured. Signs are posted.	B23										
Copy of current vaccination records for pets	C47										

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RESIDENT CARE AUDIT

Date: 1/0/00

0

RESIDENT APARTMENT NUMBER

ITEM

HEALTH SERVICES

MEDICATION/TREATMENT RECORDS

MARs match current MD orders & meds (SAMPLE)

Omission of initials <3%

Doses not given circled with reason on back

PRNs have clear parameters for use

PRNs documented on back with results

Meds are stored in original or Pharmacy packaging

Daily narc count is completed and accurate

CONSULTS/MISCELLANEOUS

Plan & visit notes for resident arranged services

Outside provider info on Service Plan

Registration and Information form (129) &

Acknowledgement and Indemnification (132) &

Waiver & Release & Insurance (133) OR

Copy of License, Insurance & Contract for Agency

ASSESSMENT/CARE PLANS

Resident Evaluation & Service Plan at move in

Required services provided (include NSA approaches)

Signed and dated by resident or legal representative

Points are scored correctly

Reflects change in condition

Reevaluated at least every 6 mo (annual for IND)

Reevaluated upon return from HLOC

Indicated level matches billing

Monthly review done

Outside Services identified

Slide rail/assistive device evaluation documented

Negotiated Service Agreement-if applicable

Resident meets MG & State guidelines for residency

GARDEN HOUSE - ADDITIONAL ITEMS

GH Notification of Risks and Benefits signed

Mini Mental Sataus Exam completed

Physical Self Maintenance Scale completed

ADDITIONAL ITEMS

Oxygen is secured. Signs are posted.

Copy of current vaccination records for pets

= Incomplete
na = Not applicable
* = Multiple items per standard

x = Complete
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[illegible]

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Merrill Gard

Audit Summary - Health Services

Type: **Draft**

Refer to page one of this work book to review the audit routine and procedure.

Community: [Specify on Chart Audit-AL]

Auditor: _____

Date: _____

[Chart Audit K1]

QTR: _____

GM: _____

RCD: _____

Tenure: _____

GHD: _____

Tenure: _____

Category A:

Acceptable Range: **100%**

High Priority: Items that are potential Life Safety Issues and / or have Significant Financial Impact.

Standard # Assessments:

Standard #	Assessments:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
1	All residents meet state guidelines and Merrill Gardens criteria for accepting and retaining residents.	OPG 1.47.3	100%			0	
2	All residents have a Resident Evaluation and Service Plan and Medical Health Statement (108 or H&P) completed before move-in.	OPG 3.4 & state requirements	100%			0	

Standard # Service Plans:

Standard #	Service Plans:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
3	Garden House residents have signed Notice of Risks and Benefits	GH 5.2	100%			0	
4	Required health services are being provided to residents. Consider services needed to prevent decline in health for purpose of determining compliance. Includes following NSA approaches.	OPG 4.47.3	100%			0	

Standard # Medications:

Standard #	Medications:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
5	Medication Assist/Administration Records match orders given by physician & meds on hand. (Sample)	OPG 21.1, 21.7, 21.8, 22.4-22.6	100%			0	

Standard # Safety/Incidents:

Standard #	Safety/Incidents:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
6	911 is called for all medical emergencies.	OPG 12.2, 13.1	100%			0	
7	Follow up to reports of potential abuse indicates steps are taken immediately upon report of abuse to protect residents from a suspected abuser.	OPG 14.1, 14.2	100%			0	
8	Chemicals & hazardous materials, such as meds are kept in direct sight of staff or in locked storage.	OPG 22.3, 30.2	100%			0	
9	Daily E-call Log is completed, reviewed & signed daily by ALS & GM or MODs. Log must include flu documentation for response times greater than 10 minutes. Test of system indicates it is working properly.	OPG 9.2	100%			0	
10	The daily resident check-in report is completed and accurate for all residents, signed daily by the responsible staff member, ALS & GM (or MODs), and kept on file.	OPG 9.1	100%			0	

Category A Totals:

Total Possible Points: **0**

Total Points: **0**

Actual Score: **0**

Merrill Gardens

Audit Summary - Health Services

Type: **Draft**

Refer to page one of this work book to review the audit routine and procedure.

Community: [Specify on Chart Audit-AL]

Auditor: _____

Date: [Chart Audit K1]

QTR: _____

GM: _____

RCD: _____

Tenure: _____

GHD: _____

Tenure: _____

Category B:

Acceptable Range: **85%**

Medium Priority: Items that are likely to impact resident's health, result in citations or have some financial impact.

Standard #	Service Plans & Logs:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
11	Care refusal trends have follow up documented.	OPG 4.7	85%			0	
12	All Service Plans are signed by the resident. Legal representative may sign if resident is unable to. Must have evidence of legal representative's right to sign for resident; i.e., POA for Healthcare or Guardianship documents on file at the community.	OPG 4.3, 50	85%			0	
13	All Services are scored correctly. Correct Service Level is assigned based on totalling scoring of agreed-upon services.	OPG 3, 4.3, 4.6	85%			0	
14	Physician and responsible party are notified of changes in condition. Notification is documented in the progress notes.	OPG 23	85%			0	
15	Residents are re-evaluated prior to or upon return from higher care setting such as a hospital or skilled nursing facility with change in condition, and semi-annually.	OPG 3, 4, 23.1	85%			0	
16	Service Levels indicated on Service Plans match Service levels being billed.	OPG 1.9, 3, 4, 6.4	85%			0	
17	Negotiated Service Agreements are completed for residents identified at risk for falls, skin breakdown or elopement.	OPG 47.1	85%			0	
18	Service Log documentation omission rate is less than 3%.	OPG 4.7	85%			0	
19	Scheduled services not provided are initialed, initials circled, and the reason not provided documented on the back of the Log.	OPG 4.7	85%			0	

Safety & Incidents:

Standard #	Safety & Incidents:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
20	Emergency Contact Information is listed.	OPG 2.2	85%			0	
21	All incidents are documented on an Incident Report form and in the Progress notes. Documentation must include Circumstances, Action taken, Notification of MD/Family (State Agencies as appropriate), and Preventative measures/follow-up. First staff member with knowledge of the incident must initiate the report.	OPG 12, 13.13	85%			0	
22	Bed rails & adaptive equipment have documented evaluation.	OPG 13.6, 48.1	85%			0	
23	Compressed gas tanks (oxygen, helium, etc.) are secured or in approved containers. Signs are posted.	OPG 45.1, 45.3	85%			0	

Merrill Gard Audit Summary - Health Services

Refer to page one of this work book to review the audit routine and procedure.

Type: **Draft**

Community: [Specify on Chart Audit-AL]

Auditor: _____

Date: _____

[Chart Audit K1]

QTR: _____

GM: _____

RCD: _____

Tenure: _____

GHD: _____

Tenure: _____

Standard # Medications:

Standard #	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
24	Medications are kept in locked storage. Narcotics are double locked.	85%			0	
25	Narcotics are counted each shift and verified by signatures of two staff members.	85%			0	
26	Medications are kept in original or pharmacy prepared container until set up for delivery.	85%			0	
27	Documentation on MARs includes Initialed meds given, refusals, and PRN follow-up. Documentation omission rate is <3%.	85%			0	
28	Sharps container is available in medication area.	85%			0	

Standard # Resident Files:

Standard #	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
29	Resident records are safeguarded against unauthorized access.	85%			0	
30	Issues identified in a progress note have documented follow up to resolution.	85%			0	
31	Private Sitters/Home Health and other outside Care Providers have completed signed documents on file prior to provision of services. Copies of Care Plan and visit notes are in chart.	85%			0	
32	State required information forms are in charts.	85%			0	

Standard # Staffing:

Standard #	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
33	Staff qualifications meet state requirements for level of service provided.	85%	x		0	
34	Care staff background screening and Tb Testing done at hire	85%	x		0	
35	Staffing levels are within budgeted hours based on census and acuity and/or meet state requirements.	85%			0	
36	State and Merrill Gardens required orientation & specialty training is completed and documented.	85%			0	
37	State & Merrill Gardens required continuing education training is completed and documented.	85%			0	

Category B Totals:

Total Possible Points: **4**

Total Points: **0**

Actual Score: **100%**

Merrill Gardens

Audit Summary - Health Services

Type: **Draft**

Refer to page one of this work book to review the audit routine and procedure.

Community: [Specify on Chart Audit-AL]

Auditor: _____

Date: _____

[Chart Audit K1]

QTR: _____

GM: _____

RCD: _____

Tenure: _____

GHD: _____

Tenure: _____

Low Priority: Non-critical issues such as paperwork not impacting resident safety, low risk of citation.

Category C: Acceptable Range: **75%**

Standard #	Assessments/Service Plans/Logs:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
38	A monthly review of all AL Service Plans is documented in the designated place on the form.	OPG 4.3	75%			0	
39	Services provided by private duty care providers/ agencies are listed on Service Plans.	OPG 4.3, 35.5	75%			0	
40	Service Log documentation matches Service Plan.	OPG 4.7	75%			0	
41	GH-Mini Mental Status Exam completed.		75%			0	
42	GH-Physical Self Maintenance Scale completed.		75%			0	

Standard #	Incidents/Safety:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
43	Completed incident reports are signed by GM and ALS and are kept in a binder in the GM office.	OPG 12, 13	75%			0	

Standard #	Medications:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
44	Physician ordered Blood pressures, pulses and blood sugars have parameters for MD notification. Physician is notified per parameters.	OPG 22.5 & 22.6	75%			0	
45	PRN medication orders have clearly defined parameters for use (dose, reason, timing)	OPG 22.8	75%			0	

Standard #	Miscellaneous:	Policy Reference	Acceptable Compliance %	Yes	No	Points	# of times cited
46	Operations Manual has current revision & is located where it can be accessed by caregivers 24/7. Current state specific policies and forms are in manual.	OPG Purpose & State Requirements	75%			0	
47	Pets have documentation of current vaccinations	OPG 43.4	75%			0	

Category C Totals:

Total Possible Points: **0**

Total Points: **0**

Actual Score: **0**

Merrill Gard
Audit Summary - Health Services

Type: **Draft**

Refer to page one of this work book to review the audit routine and procedure.

Community: [Specify on Chart Audit-AL]

Auditor: _____

Date: _____

[Chart Audit K1]

QTR: _____

GM: _____

RCD: _____

Tenure: _____

GHD: _____

Tenure: _____

Category	Acceptable Range	Actual Score
A	100%	
B	85%	
C	75%	100%

Total Score **4** of **4**
100%

Worksheet Not Completed

Merrill Gardens

Health Services Action Plan and Correction Summary

Community: [Specify on Resident Audit D1] Auditor: [Specify on Audit Summary] Date: [Chart Audit K1]

Observation			Action	By Whom	Due Date	Date Completed
Standard						
Policy						
Category/Cite #						Completion Confirmed By
Standard						
Policy						
Category/Cite #						Completion Confirmed By
Standard						
Policy						
Category/Cite #						Completion Confirmed By
Standard						
Policy						
Category/Cite #						Completion Confirmed By

Merrill Gardens

Health Services Action Plan and Correction Summary

Community: [Specify on Resident Audit D1] Auditor: [Specify on Audit Summary] Date: [Chart Audit K1]

Observation			Action		By Whom	Due Date	Date Completed
Standard							
Policy							Completion Confirmed By
Category/Cite #							
Standard							
Policy							Completion Confirmed By
Category/Cite #							
Standard							
Policy							Completion Confirmed By
Category/Cite #							
Standard							
Policy							Completion Confirmed By
Category/Cite #							
Standard							
Policy							Completion Confirmed By
Category/Cite #							

Community: [Specify on Resident Audit D1] Auditor: [Specify on Audit Summary H4] Date: [Chart Audit K1]

Survey Issue Follow-up:

List any Survey issues since last audit and whether corrected. Uncorrected issues are documented on Action Plan and considered as repeat issues.

Residents' Comments:

Compliments and concerns expressed by residents.

MG Cares Comments:

Describe your impressions of the community as though you were a customer.

- | | | |
|------------------------------|-----------------------------|--|
| ☐ Yes | ☐ No | Were you greeted appropriately when entering the community? |
| ☐ Yes | ☐ No | Were name tags worn by all team members? |
| ☐ Yes | ☐ No | Was team member appearance consistently neat, wearing appropriate MG shirt, shoes, etc.? |
| ☐ Yes | ☐ No | Did team members address residents by name, make eye contact, listen and smile? |
| ☐ Yes | ☐ No | Did you observe residents or guests being greeted when entering the community/approaching the front desk/dining room in a timely manner? |
| ☐ Yes | ☐ No | Were the common areas clean and free of debris? |
| ☐ Yes | ☐ No | Were the grounds entering the community pleasant (i.e., attractive landscaping)? |
| ☐ Yes | ☐ No | Was the dining room clean? |
| ☐ Yes | ☐ No | ☐ N/A If staying at the community, was the apartment ready (set up with linens, towels, etc., and clean)? |

Other Observations/Comments:

Observations and comments regarding customer service or other areas not included elsewhere in audit findings.

8.2 Regulatory Authority Inspection

Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

All or part of a Merrill Gardens Community may be licensed as an Assisted Living Facility, or an equivalent term in the state in which it operates. This license allows the assistance of residents with certain activities of daily living (ADL's). If the Community was not licensed, performing these services would be prohibited. The license requires certain standards be met regarding resident care and operations. Several regulatory agencies inspect the Community routinely or intermittently. These may include the State Licensing Agency, Fire Marshal, Health Department and others.

Guidance

Licensure of Merrill Gardens Communities

Merrill Gardens is committed to operating in a manner that promotes the health, safety, and quality of life of our residents. The Community is required to abide by and operate in accordance with the rules and regulations set forth by the state, as well as those specified by Merrill Gardens. All management staff and key personnel of the Community should read and be familiar with the regulations.

STATE LICENSING AUTHORITY

The State Licensing (Authority) assigns an inspector to oversee the Community operations and to verify that it is operating in accordance with state regulations. The inspector usually visits the Community at least once per year to analyze the operation and make determinations regarding compliance.

Generally, inspectors are required to advise the Community of the purpose of their visit, and to do an exit interview. They will leave a written report stating the purpose of their visit, their observations and any areas that need correction. If there are any deficiencies, they will also give a time frame in which to correct these problems. A copy of this report should be sent to the Vice President of Operations, Vice President of Resident Care and the Regional Director of Health Services within 24 hours of the inspection.

What to Do During an Inspection

A representative of the regulatory authority, referred to as inspector, may enter the building at any time and inspect the premises. Visits are generally unannounced. If this happens, please do the following:

1. Politely ask the inspector to show identification.
2. Summon the General Manager immediately. Ask the inspector to wait for the General Manager prior to beginning the inspection. The inspector does not have to wait, but ask anyway.

3. Alert all Community Department Heads of the inspection. Inspectors may notify other agencies if they have concerns outside of their scope of authority, so all departments need to be aware of the inspection in process. All Community Department Heads should remain on the premises for the duration of the inspection unless released by the General Manager.
4. The General Manager should accompany the inspector on any tour of the Community. The General Manager may assign the appropriate Community Department Head to participate in the tour as they may have more detailed knowledge.

Inspection Communication Process

Inspection Process Initiation

Inspector makes contact with the Community by phone, visit, etc.

1. General Manager or designated person calls the Vice President of Operations. The Vice President of Operations will notify the appropriate Regional or Seattle office Department Heads. This will put these resources on alert in case their assistance is needed during the process.

Inspection Exit

1. Inspector informs General Manager of outcome of visit.
2. General Manager reports findings to Vice President of Operations in verbal and written form.
3. Vice President of Operations contacts the appropriate Regional or Seattle office Department Heads to discuss exit findings.
4. General Manager scans copy of inspection report to Vice President of Operations and Regional Director of Health Services when received.
5. Regional Director of Health Services scans a copy of the inspection report to Vice President of Resident Care and Vice President of Health Services. Vice President of Health Services will forward to appropriate Seattle office departments.

Follow-up

1. A plan of correction may be required to be submitted. The Vice President of Operations and Regional Director of Health Services should work with the GM to develop and review the plan before it is submitted to the regulatory agency.
2. Vice President of Operations communicates with General Manager to verify that corrections and timelines are on track.
3. Vice President of Operations consults with appropriate Regional Director of Health Services and Seattle office Department Head.
4. Vice President of Operations reports any follow-up information to Vice President of Health Services.

SC: *The Community will submit an acceptable Plan of Correction to the Department signed by the administrator and returned by the date specified. The plan is to include: Actions taken to correct*

each cited deficiency, Actions taken to prevent recurrences of actual and similar deficiencies, and the actual or expected completion date of the actions.

Reports of inspections, including the Community response, will be provided to the public upon written request with the redaction of the names of individuals in the report.

WA: *The results of the most recent licensing inspection must be kept in a location accessible to the public.*



Quality Assurance and Oversight

Resident Care Manual

8.3 Ombudsman Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Ombudsman programs exist to protect the rights of residents and to help ensure quality Community care. Merrill Gardens supports these programs as a potentially helpful resource in addressing resident issues. Cooperation with Ombudsman is encouraged.

Definition

Ombudsman

State Agency representative advocating residents' rights.

Guidance

Ombudsman

Merrill Gardens refers to the representative of the Ombudsman's office as the ombudsman. It is recognized that this person may be either male or female in this reference.

In states with an Ombudsman program, the Community is assigned an Ombudsman representative. This person works with the state and many community organizations to help ensure that the health, safety, quality of life and personal rights of the residents are not being compromised. They are the residents' advocate and can often be of assistance to the Community when dealing with an issue regarding a resident.

Remember:

- An Ombudsman should always be treated with respect.
- An Ombudsman is only authorized to review files, and Community documentation that pertain to the specific issue or complaint being investigated. Resident permission must be obtained in accordance with state laws to allow access to confidential resident information.

If questions arise regarding the Community's Ombudsman or how they can assist, contact the local Ombudsman office or the Vice President of Operations.

8.4 Resident Councils and Committees

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens actively supports the ongoing activity of a Resident Council. The Community facilitates Resident Council meetings monthly or as scheduled by the Council. The purpose of these meetings is to discuss issues, improve communication, and to work together to improve the quality of life for all residents. The Resident Council meeting is conducted in accordance with applicable state laws. Additionally, as appropriate, the Community will establish and participate in Resident Committees.

CA: Under California law, every licensed residential care facility for the elderly, at the request of two or more residents, is required to assist the residents in establishing and maintaining a resident council. Merrill Gardens has assisted in the establishment of a resident council.

SC: Residents are provided the opportunity to provide input into changes in community operational policies, procedures, and services including House Rules through the Resident Council.

Guidelines

Resident Councils

Resident Council meetings are run by a board of residents elected to office by the residents of the Community. The board consists of a President, Vice President and a Recording Secretary. Elections should be held at least annually.

- The Resident Council is composed of residents of the Community. At the invitation of the Resident Council, family members, resident representatives, advocates, long-term care ombudsman program representatives, community staff, or others may participate in Council meetings and activities.
- The Resident Council may, among other things, make recommendations to the General Manager to improve the quality of daily living and care in the Community and to promote and protect residents' rights. The Community management team has an obligation to listen to the residents' comments and concerns, but has the final decision in matters of Community operations, staffing, marketing, etc.
- If the Resident Council submits written concerns or recommendations, The General Manager or designee will respond in writing within 14 calendar days regarding any action or inaction taken as a result.

- Merrill Gardens will not limit the right of residents to meet independently with outside persons or with the Community's personnel.
- Merrill Gardens will inform each Resident Council member of his or her right to be interviewed as part of the regulatory inspection process.
- Merrill Gardens will promote the Resident Council as follows:
 - a. Merrill Gardens will appoint a designated staff liaison at the Community to assist the Resident Council.
 - b. Merrill Gardens will make a room available at the Community for Resident Council meetings and will post meeting information in a central location at the Community readily accessible to residents, relatives, and resident representatives. Notices for the meetings will be posted in the Activity Calendar. Minutes of the meetings should be made available for all residents to review.
 - c. Upon request, and with the permission of the Resident Council, Merrill Gardens will share the name and contact information of the designated representative of the Resident Council with the long-term care ombudsman program.

CA: *Merrill Gardens will inform new residents of the existence of the Resident Council in the Resident Agreement. Merrill Gardens also will provide written information at the time of admission on the time, place, and dates of Resident Council meetings and the resident representative to contact regarding involvement in the Resident Council.*

- Merrill Gardens will not willfully interfere with the formation, maintenance, or promotion of the Resident Council, or its participation in the regulatory inspection process. This includes, but is not limited to, discrimination or retaliation in any way against an individual as a result of his or her participation in a Resident Council, refusal to publicize Resident Council meetings or provide appropriate space for either meetings or a bulletin board, or failure to respond to written requests by the Resident Council in a timely manner.

CA: *Merrill Gardens will post the text of the statutory provision setting forth its obligations with respect to the Resident Council (Cal. Health & Safety Code §1569.157) with the heading "Rights of Resident Councils" in a prominent place at the Community accessible to residents, family members, and resident representatives.*

Sample Format of General Resident Council Meeting

1. President opens the meeting.
2. Recording Secretary reads the minutes from the last meeting.
3. President and General Manager report on progress or resolution of any issues discussed at the last meeting.
4. President leads discussion on new business, following the agenda drawn up by the Council and the General Manager.
5. President asks if there is any further business.

6. Further business is discussed.
7. President adjourns the meeting.

It is the President's responsibility to maintain order and keep the meeting on track.

Committees

FOOD SERVICE COMMITTEE

Resident committee members participate in meetings to discuss food service issues with the Food Service Manager and the General Manager.

OTHER COMMITTEES

Committees will be set up at the discretion of the General Manager and/or Resident Council to address specific Community and/or resident needs and interests.



8.5 Review of Policies and Procedures

Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Merrill Gardens strives to provide current guidance to team members in order to perform their duties. Policies and procedures will be reviewed to ensure they meet or exceed, but do not interfere with regulatory requirements.

Guidelines

Policies and procedures should routinely be reviewed annually.

The review should be completed by a Merrill Gardens corporate team member in the related department. It is preferred that a workgroup of team members familiar with the subject matter is involved in the review process.

When regulations are changed or newly implemented, an interim review of applicable policies should be done by the General Manager to ensure any needed revisions are made. The General Manager should make the appropriate corporate team member aware of any regulatory updates needed.

Documentation

The review should be documented on the *Policy-Procedure Manual Review form* and retained in the General Manager's office. The reviewer should sign and date when the review is completed.



DATE OF REVIEW	REVIEWER (print name)	SIGNATURE	TITLE
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[illegible]

